

Ponatinib / Asciminib nella LMC – Newsletter interna (ultimi 15 giorni)

Generata il 01/10/2026

Generata il: 2026-10-01

Newsletter interna – sintesi operativa

Messaggi chiave (da leggere in 60 secondi):

- Nel periodo emergono soprattutto temi su **Resistenza / T315I / mutazioni** (n=3).
- Secondo filone: **Sequencing / linee di terapia** (n=3).
- Terzo filone: **Real-world (utilità pratica)** (n=2).

Cosa cambia per noi (azioni pratiche):

- Rivedere/rafforzare **monitoraggio cardiovascolare e interazioni** (es. anticoagulanti) nei pazienti in TKI.
- Valutare criteri interni per **consolidamento / de-escalation / TFR** nei pazienti in deep response.
- Verificare percorso di **test mutazionale** e criteri di switch nelle resistenze (incl. T315I).

Safety radar (segnali/interazioni da monitorare):

- Monitorare evoluzione su **TFR / stop terapia** (nuovi segnali/dati).
 - Monitorare evoluzione su **Safety e comorbidità** (nuovi segnali/dati).
 - Monitorare evoluzione su **Resistenza / T315I / mutazioni** (nuovi segnali/dati).
-

Top 5 da leggere subito (ordinati per impatto)

1) Integrating Asciminib into the Evolving Treatment Landscape of Chronic Myeloid Leukemia: A Canadian Perspective.

Review*Journal of blood medicine*

Score: 10

Focus clinico: Resistenza / T315I / mutazioni, TFR / stop terapia, Sequencing / linee di terapia, Real-world (utilità pratica), Sintesi (review/linee guida)

PMID: 42807211 | 10.2147/JBM.S627651

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42807211/>

- **Tipo di studio/popolazione:** Five years after the first readout of the ASCEMBL trial, asciminib evidence has rapidly expanded across the chronic myeloid leukemia (CML) treatment landscape.
- **Risultato chiave:** In this expert opinion review, we synthesize emerging clinical and real-world evidence for asciminib and discuss practical considerations for its integration into contemporary CML management, including its potential implications for TFR eligibility.
- **Rilevanza clinica:** Collectively, these data position asciminib as an established later-line therapy and an emerging frontline option, with the potential to improve long-term treatment sustainability through favorable tolerability and early molecular response kinetics.

2) Characterizing bleeding adverse events associated with BCR-ABL tyrosine kinase inhibitors: insights from FAERS reports.

Real-world / osservazionale*Frontiers in oncology*

Score: 10

Focus clinico: Safety e comorbidità, Real-world (utilità pratica)

PMID: 42780512 | 10.3389/fonc.2026.1912458

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42780512/>

- **Tipo di studio/popolazione:** This study aimed to characterize bleeding-related adverse event signals associated with five BCR-ABL TKIs using the U.S.
- **Risultato chiave:** Among 20, 326, 782 deduplicated FAERS reports, 124, 797 reports involving BCR-ABL TKIs were identified, including 3, 648 bleeding cases and 121, 149 non-bleeding cases.
- **Rilevanza clinica:** A total of 64 positive bleeding-related signals were detected across the five study drugs.

3) Allogeneic Hematopoietic Stem Cell Transplantation Achieves Durable Remission in Blast-Phase Chronic Myeloid Leukemia Presenting as Early T-Cell Precursor Acute Lymphoblastic Leukemia.

Trial clinico

EJHaem

Score: 9

Focus clinico: Resistenza / T315I / mutazioni, Sequencing / linee di terapia

PMID: 42780687 | 10.1002/jha2.70405

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42780687/>

- **Tipo di studio/popolazione:** The presentation of chronic myeloid leukemia (CML) in the blast phase as early T-cell precursor acute lymphoblastic leukemia (ETP-ALL) is very rare.
- **Risultato chiave:** We encountered a patient with blast phase CML presenting as ETP-ALL.
- **Rilevanza clinica:** HSCT following remission induction therapy, inclusive of TKIs, may enhance prognosis for patients with blast phase CML presenting as ETP-ALL.

4) Dual-Driver Myeloproliferative Neoplasm: Concurrent p190 *BCR::ABL1* CML and *JAK2* V617F Mutation: A Case Report and Literature Review.

Review

Case reports in hematology

Score: 4

Focus clinico: Resistenza / T315I / mutazioni, Safety e comorbidità, Sequencing / linee di terapia, Sintesi (review/linee guida)

PMID: 42781115 | 10.1155/crh/2927231

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42781115/>

- **Tipo di studio/popolazione:** The concurrent occurrence of *BCR::ABL1* rearrangement and *JAK2* V617F mutation is an exceptionally rare phenomenon, with reported frequencies of 0.2%-2.5% in tested myeloproliferative neoplasm (MPN) cohorts and an estimated overall rate of approximately 0.4% in patients screened for both alterations.
- **Risultato chiave:** These molecular drivers are generally considered mutually exclusive, as *BCR::ABL1* defines Philadelphia chromosome-positive (Ph+) chronic myeloid leukemia (CML), while *JAK2* V617F characterizes Ph-negative MPNs.
- **Rilevanza clinica:** The patient received first-line asciminib and was subsequently transitioned to dasatinib, with the *BCR::ABL1*/*ABL1* ratio declining from 88.5% at diagnosis to 0.79% at most recent assessment.

5) A Case of Duffy-Null-Associated Neutropenia in a Patient With Chronic Myeloid Leukemia.

Preclinico / laboratorio

Case reports in hematology

Score: 2

Focus clinico: Dose strategy / consolidamento

PMID: 42801051 | 10.1155/crh/9335086

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42801051/>

- **Tipo di studio/popolazione:** These patients do not have an increased risk of infection and may have an ANC $< 1.5 \times 10^3$ /L at baseline.
- **Risultato chiave:** After several treatment changes due to adverse effects, she began treatment with asciminib 80 mg daily.
- **Rilevanza clinica:** BCR-ABL1 transcript level with asciminib was consistently below 0.1%, indicating successful treatment response.

Evidenze per domande cliniche

Resistenza / T315I / mutazioni (3)

Score: 0

Mini-riassunto (da abstract, estrattivo):

Riassunto aggiuntivo (full text Open Access, estrattivo):

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Review

Journal of blood medicine

Score: 10

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- **Rilevanza clinica:** Collectively, these data position asciminib as an established later-line therapy and an emerging frontline option, with the potential to improve long-term treatment sustainability through favorable tolerability and early molecular response kinetics.
- **Metodi (full text OA):** Evidence: The Phase 3 ASCEMBL trial established asciminib as an effective option for patients with chronic-phase CML previously treated with at least two TKIs.^{15–17} In this randomized study, asciminib demonstrated superiority to bosutinib for major molecular response (MMR) at week 24 (25.5% vs 13.2%; primary endpoint) and continued deepening of responses with longer follow-up (37.6% vs 15.8% at week 96).^{15,16} Molecular responses were generally durable once achieved, and treatment discontinuations due to AEs were substantially lower with asciminib than with bosutinib (8.3% vs 27.6% at week 156).¹⁷ Consistent efficacy and tolerability of asciminib have also been reported in Canadian real-world cohorts of heavily pretreated patients, with one-year MMR rates of approximately 42%.²³ The safety profile of asciminib in later-line settings is characterized primarily by hematologic toxicities, most commonly thrombocytopenia (grade ≥ 3 : 22.4%; all grades: 29.5%), and less frequently elevations in pancreatic enzymes (lipase grade ≥ 3 : 3.8%, all grades: 5.1%; amylase grade ≥ 3 : 0.6%, all grades: 5.8%).¹⁷ These events typically occur early within the first weeks to months of treatment initiation and are generally manageable with dose interruption or modification.
- **Risultato principale (full text OA):** non riportato.
- **Dettaglio utile (full text OA):** non riportato.
- **Messaggio clinico (full text OA):** Asciminib represents an important advance in CML therapy as a mechanistically distinct option that provides effective molecular responses and a favorable safety profile compared with ATP-site TKIs.

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Score: 9

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Review

Case reports in hematology

Score: 4

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Sequencing / linee di terapia (3)

Score: 0

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Real-world (utilità pratica) (2)

Score: 0

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Review

Journal of blood medicine

Score: 10

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Real-world / osservazionale

Frontiers in oncology

Score: 10

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- **Rilevanza clinica:** A total of 64 positive bleeding-related signals were detected across the five study drugs.
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Sintesi (review/linee guida) (2)

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Open Access (Europe PMC/PMC): sì (PMC13598437)

- **Tipo di studio/popolazione:** The concurrent occurrence of <i>BCR::ABL1</i> rearrangement and <i>JAK2</i> V617F mutation is an exceptionally rare phenomenon, with reported frequencies of 0.2%-2.5% in tested myeloproliferative neoplasm (MPN) cohorts and an estimated overall rate of approximately 0.4% in patients screened for both alterations.
- **Risultato chiave:** These molecular drivers are generally considered mutually exclusive, as <i>BCR::ABL1</i> defines Philadelphia chromosome-positive (Ph+) chronic myeloid leukemia (CML), while <i>JAK2</i> V617F characterizes Ph-negative MPNs.
- **Rilevanza clinica:** The patient received first-line asciminib and was subsequently transitioned to dasatinib, with the <i>BCR::ABL1</i>/<i>ABL1</i> ratio declining from 88.5% at diagnosis to 0.79% at most recent assessment.
- **Metodi (full text OA):** non riportato.
- **Risultato principale (full text OA):** non riportato.
- **Dettaglio utile (full text OA):** non riportato.
- **Messaggio clinico (full text OA):** Any biological rationale, such as the reliance of p190 on Src family signaling that dasatinib but not asciminib targets, remains speculative and requires formal study [15, 16, 20, 21].

TFR / stop terapia (1)

Score: 0

Mini-riassunto (da abstract, estrattivo):

Riassunto aggiuntivo (full text Open Access, estrattivo):

Integrating Asciminib into the Evolving Treatment Landscape of Chronic Myeloid Leukemia: A Canadian Perspective.

Review

Journal of blood medicine

Score: 10

PMID: 42807211 | 10.2147/JBM.S627651

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42807211/>

Open Access (Europe PMC/PMC): sì (PMC13617284)

- **Tipo di studio/popolazione:** Five years after the first readout of the ASCEMBL trial, asciminib evidence has rapidly expanded across the chronic myeloid leukemia (CML) treatment landscape.
- **Risultato chiave:** In this expert opinion review, we synthesize emerging clinical and real-world evidence for asciminib and discuss practical considerations for its integration into contemporary CML management, including its potential implications for TFR eligibility.
- **Rilevanza clinica:** Collectively, these data position asciminib as an established later-line therapy and an emerging frontline option, with the potential to improve long-term treatment sustainability through favorable tolerability and early molecular response kinetics.
- **Metodi (full text OA):** Evidence: The Phase 3 ASCEMBL trial established asciminib as an effective option for patients with chronic-phase CML previously treated with at least two TKIs.^{15–17} In this randomized study, asciminib demonstrated superiority to bosutinib for major molecular response (MMR) at week 24 (25.5% vs 13.2%; primary endpoint) and continued deepening of responses with longer follow-up (37.6% vs 15.8% at week 96).^{15,16} Molecular responses were generally durable once achieved, and treatment discontinuations due to AEs were substantially lower with asciminib than with bosutinib (8.3% vs 27.6% at week 156).¹⁷ Consistent efficacy and tolerability of asciminib have also been reported in Canadian real-world cohorts of heavily pretreated patients, with one-year MMR rates of approximately 42%.²³ The safety profile of asciminib in later-line settings is characterized primarily by hematologic toxicities, most commonly thrombocytopenia (grade ≥ 3 : 22.4%; all grades: 29.5%), and less frequently elevations in pancreatic enzymes (lipase grade ≥ 3 : 3.8%, all grades: 5.1%; amylase grade ≥ 3 : 0.6%, all grades: 5.8%).¹⁷ These events typically occur early within the first weeks to months of treatment initiation and are generally manageable with dose interruption or modification.
- **Risultato principale (full text OA):** non riportato.
- **Dettaglio utile (full text OA):** non riportato.
- **Messaggio clinico (full text OA):** Asciminib represents an important advance in CML therapy as a mechanistically distinct option that provides effective molecular responses and a favorable safety profile compared with ATP-site TKIs.

Dose strategy / consolidamento (1)

Score: 0

Mini-riassunto (da abstract, estrattivo):

Riassunto aggiuntivo (full text Open Access, estrattivo):

A Case of Duffy-Null-Associated Neutropenia in a Patient With Chronic Myeloid Leukemia.

Preclinico / laboratorio

Case reports in hematology

Score: 2

PMID: 42801051 | 10.1155/crh/9335086

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42801051/>

Open Access (Europe PMC/PMC): sì (PMC13615517)

- **Tipo di studio/popolazione:** These patients do not have an increased risk of infection and may have an ANC $< 1.5 \times 10^3$ /L at baseline.
- **Risultato chiave:** After several treatment changes due to adverse effects, she began treatment with asciminib 80 mg daily.
- **Rilevanza clinica:** BCR-ABL1 transcript level with asciminib was consistently below 0.1%, indicating successful treatment response.
- **Metodi (full text OA):** non riportato.
- **Risultato principale (full text OA):** non riportato.
- **Dettaglio utile (full text OA):** non riportato.
- **Messaggio clinico (full text OA):** In the treatment of CML, the goal of treatment is achieving major molecular response, which is BCR-ABL1 transcript level $< 0.1\%$ by 12 months [6].